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Specialty Medications for the Treatment of Inflammatory Conditions Related to Skin and Joints for Non-Medicare Members

DESCRIPTION

This policy addresses specialty injectable agents for the management of inflammatory medical conditions related to skin and joints. Coverage for oral agents is guided by the Formulary listing (quantity limits, step therapy, Tier structure, prior authorization, specialty) and the HAP Criteria for Use of Drugs document.

Please Note: The following list(s) of procedure and/or diagnosis codes is provided for reference purposes only and may not be all inclusive. Listing of a code in this policy does not imply that the service described by the code is a covered or non-covered health service. Benefit coverage for health services is determined by the member specific benefit plan document and applicable laws that may require coverage for a specific service. The inclusion of a code does not imply any right to reimbursement or guarantee claim payment. Other Policies and Guidelines may apply.

RELEVANT HCPCS CODES

J0129	Injection, abatacept, 10 mg
J0638	Injection, Canakinumab, 1 mg
J0717	Injection, certolizumab pegol, 1 mg
J1438	Injection, etanercept, 25 mg
J1628	Injection, guselkumab, 1 mg
J1747	Injection, spesolimab-sbzo, 1 mg
J3245	Injection, tildrakizumab, 1 mg
J3262	Injection, tocilizumab, 1 mg
J3357	Ustekinumab, for subcutaneous injection, 1 mg
Q5133	Injection, tocilizumab-bavi (tofidence), biosimilar, 1 mg
Q5135	Injection, tocilizumab-aazg (tyenne), biosimilar, 1 mg
Varies	See Biosimilar and Preferred Formulations for: infliximab, or rituximab

COVERAGE CRITERIA

Coverage Criteria, Limitations, and Exclusions for use of the following specialty medications is outlined below: Etanercept (Enbrel®), Certolizumab Pegol (Cimzia®), Golimumab (Simponi®, Simponi Aria®), Abatacept (Orencia®), Tocilizumab (Actemra®), Anakinra (Kineret®), Ustekinumab (Stelera®), Secukinumab (Cosentyx®), Sarilumab (Kevzara®), Guselkumab (Tremfya®), Brodalumab (Siliq®), Tildrakizumab (Ilumya®), Spesolimab-sbzo (Spevigo®). Coverage criteria will also apply to specialty medications with a biosimilar product (unless otherwise noted in the Policy).

GENERAL COVERAGE CRITERIA: Applicable to all medication requests.

1. Member must be 18 years of age or older (unless noted otherwise in coverage criteria or FDA-approved prescribing information for a specific medication and indication).
2. Must be ordered by a HAP/AHL Affiliated or Contracted Dermatologist, Rheumatologist, Pulmonologist, Ophthalmologist (depending on the requested indication)
3. As applicable per prescribing information regarding tuberculosis assessment for specialty medication: The Member must have tuberculin skin test (TST) or interferon-γ-release assay (IGRAs) completed within 1 year prior to initiation of biologic therapy. Exposure to BCG vaccination causes high incidence of false positive TST result; these members should receive IGRAs (QFT-GIT or T-spot).
 - a. Negative test result: retest annually for Members who live, travel, or work in situations where TB exposure is likely. Monitor Member for clinical signs and symptoms of TB.
 - b. Positive test result:
 - i. Member must have chest radiograph and if suggestive of active TB, a subsequent sputum analysis to confirm active TB.
 - ii. Completion of treatment with antitubercular medications is required prior to initiation or resumption of biologic therapy.
4. Criteria will apply for IV formulations and self-administered medications.
5. All information necessary for clinical review of the request is required: medical records, including rationale for medication use, previous therapies used, relevant laboratory and diagnostic tests and duration of requested therapy.

PLAQUE PSORIASIS

Plaque psoriasis is the most common form of psoriasis and is characterized by raised, red patches covered with a silvery white scale. Plaques commonly appear on the scalp, knees, elbows, and lower back. Treatments can be used alone (monotherapy) or

in combination (dual therapy) and include: topical agents (with or without steroids), phototherapy, and systemic medications (traditional orals and biologics).

1. Medical record must show documentation of plaque psoriasis, characterized as moderate to severe. Quantitatively, 10% or greater of body surface is covered with psoriasis plaques.
2. Documentation of:
 - a. Three month trial of any ONE of the following oral agents: methotrexate, cyclosporine or acitretin (Soriatane®) OR
 - b. Member has not responded to at least one 30-treatment course of photochemotherapy [psoralen plus ultraviolet A (PUVA)] – or – phototherapy [ultraviolet B (UVB)]. Poor response is defined as <50% clearing of psoriasis plaques from the initially affected body surface.
 - i. National Psoriasis Foundation does not support the use of indoor tanning beds as a substitute for phototherapy performed under a doctor's supervision; therefore, use of tanning beds as documentation of phototherapy does not meet this element of the criteria. <https://www.psoriasis.org/about-psoriasis/treatments/statement-on-tanning-beds>
3. Unless otherwise noted on the Formulary as covered without a prior authorization, coverage of HAP preferred anti-TNF medications (e.g.; etanercept, list not all inclusive) require coverage criteria 1 and 2 to be met.
4. For Brodalumab:
 - a. Coverage Criteria 1 through 2 are applicable; however, member must demonstrate failure of both oral agent AND phototherapy (unless medical contraindications are documented).
 - b. Member must fail a therapeutic trial of ALL HAP preferred anti-TNF agents (unless clinically significant medical contraindications are documented).
 - i. Failure of TNF inhibitor treatment is defined as a minimum of three months trial with <50% clearing of psoriasis plaques from the initially affected body surface.
 - c. Member must fail a therapeutic trial (3 months) of oral apremilast (Specific prior authorization coverage applies. Refer to Pharmacy Care Management Prior Authorization Criteria Document).
 - d. Members with Crohn's Disease are excluded from use.
 - e. Members with a history of with a history of depression and/or suicidal ideation or behavior are excluded from use.
 - f. Prior to initiation of treatment, members shall go through a screening with a mental health professional for signs and/or symptoms of depression and/or suicidal ideation or behavior.
 - g. Prescriber and member are to adhere to Siliq REMS (Risk Evaluation and Mitigation Strategy) program, as directed by the FDA.
5. For Secukinumab, Guselkumab, Tildrakizumab and Ustekinumab:
 - a. Coverage Criteria 1 through 2 are applicable; however, member must demonstrate failure of both oral agent AND phototherapy (unless clinically significant medical contraindications are documented).
 - b. Member must fail a therapeutic trial of ALL HAP preferred anti-TNF agents (unless clinically significant medical contraindications are documented).
 - i. Failure of TNF inhibitor treatment is defined as a minimum of three months trial with <50% clearing of psoriasis plaques from the initially affected body surface.
 - c. Member must fail a therapeutic trial (3 months) of oral apremilast (Specific prior authorization coverage applies. Refer to Pharmacy Care Management Prior Authorization Criteria Document).
 - d. The most cost-effective treatment course (taking into consideration requested indication, dosing, and weight) among the specialty medications (intravenous or subcutaneous) on Formulary will apply.
 - i. Additional requirement for Ustekinumab: After initial induction of ustekinumab, dosing of 45 mg every 12 weeks. For patients over 100 kg, if 45 mg dose is showing clinical improvement, but not remission after 12 weeks of treatment: 90 mg every 12 weeks. After induction, dosing intervals less than every 12 weeks (maintenance phase) are not approved.
6. For Certolizumab:
 - a. Coverage Criteria 1 through 2 are applicable; however, member must demonstrate failure of both oral agent AND phototherapy (unless clinically significant medical contraindications are documented).
 - b. Certolizumab is a non-preferred medication for treatment of moderate-to-severe plaque psoriasis; therefore, all other specialty medications listed in this Policy (specific to plaque psoriasis) must be tried (3 months) and failed) (unless clinically significant medical contraindications are documented).
 - c. 400 mg (given as two subcutaneous injections of 200 mg each) initially and at Weeks 2 and 4, followed by 200 mg every other week. If 200 mg every other week for 12 weeks is showing clinical improvement, but not remission, 400 mg every other week may be considered.

GENERALIZED PUSTULAR PSORIASIS

Generalized pustular psoriasis (GPP) is a severe and rare form of psoriasis characterized by widespread areas of inflamed skin with numerous pus-filled blisters (pustules), often accompanied by fever and systemic symptoms. Its exact cause is not fully understood but is thought to involve genetic predisposition and immune system dysfunction, triggered by factors like infections or medications. Management typically involves a multidisciplinary approach with dermatologists and other specialists, employing treatments such as topical therapies, systemic medications, phototherapy, and supportive care to alleviate symptoms and improve quality of life.

1. For Spesolimab-sbzo, the member must meet ALL of the following criteria:
 - a. Member has confirmed diagnosis of generalized pustular psoriasis (GPP) based on BOTH of the following:
 - i. Presence of primary, sterile, macroscopically visible pustules on non-acral skin; AND
 - ii. Pustulation is NOT restricted to psoriatic plaques (occurs outside of psoriatic plaques).
 - b. Member is experiencing a moderate to severe GPP flare and meets ALL of the following:
 - i. Generalized Pustular Psoriasis Physician Global Assessment (GPPPGA) total score ≥ 3 (moderate); [see Appendix below] AND
 - ii. GPPPGA pustulation subscore ≥ 2 (mild); AND
 - iii. Erythema and pustules cover $\geq 5\%$ of body-surface area; AND
 - iv. New appearance or worsening of pustules.
2. Member has tried and failed ALL of the following (unless clinically significant medical contraindications are documented):
 - a. Adequate trial (e.g., 2-4-weeks) of one VERY-HIGH potency topical steroid;
 - b. 6-week trial of Tacrolimus 0.1% ointment;
 - c. 30-treatment course psoralen plus ultraviolet A (PUVA) photochemotherapy;
 - d. 12-week trial of all of the following:
 - i. One oral immunosuppressant (e.g., cyclosporine or methotrexate)
 - A. HAP prefers the use of cyclosporine given the onset of action for management of GPP
 - ii. One oral retinoid (e.g., acitretin)
 - iii. Topical or oral dapsone
 - iv. A HAP preferred biologic (e.g., infliximab, adalimumab)
3. Member is not receiving spesolimab-sbzo in combination with any other systemic/biologic immunomodulatory specialty medications (including but limited to): anti-TNF medications, IL-inhibitor biologic medications, janus kinase inhibitor, phosphodiesterase 4 inhibitors).
4. Member does not have a diagnosis of plaque psoriasis, atopic dermatitis, hidradenitis suppurativa, palmoplantar pustulosis.
5. For treatment of GPP flares, the most cost-effective formulation of Spesolimab-sbzo will be covered (intravenous vs. subcutaneous).
6. For management of GPP when not experiencing a flare, the following criteria must be met:
 - a. Member must have met criteria 1-5.
 - b. Member must have documentation of severe GPP warranting prevention with one of the following:
 - i. Recurrent GPP flares (e.g., 4 or more flares per year) OR
 - ii. Persistent GPP flares lasting longer than 3-months.
7. Dosing limitation: see Generalized Pustular Psoriasis under Limitations for coverage details regarding dosing.

POLYMYALGIA RHEUMATICA

Polymyalgia rheumatica (PMR) is an inflammatory rheumatic condition and can be associated with giant cell arteritis (GCA) with a shared disease process. Common symptoms of PMR include morning stiffness and aching surrounding the shoulders, hip girdle and neck. The primary goal of treatment is relief of symptoms and initial treatment is low-dose glucocorticoids.

1. Member is diagnosed with Polymyalgia rheumatica (PMR) with the following: [NOTE: A current or prior diagnosis of GCA can support a diagnosis of PMR]
 - a. Age 50 years or older at onset of symptoms
 - b. Bilateral shoulder and/or hip girdle pain
 - c. Morning stiffness lasting greater than 45 minutes in at least two areas (neck or torso, shoulders or proximal regions of the arms, and hips or proximal aspects of thighs)
 - d. Duration of pain greater than 2 weeks
 - e. Elevated acute phase reactants (ESR and/or CRP)
 - f. Exclusions of mimicking conditions during the follow-up period and appropriate laboratory testing (e.g.; fibromyalgia, osteoarthritis, myositis, rheumatoid arthritis, spondyloarthropathy, etc.)
2. For sarilumab, Member must have failed at least 12 weeks of glucocorticosteroids (e.g. prednisone or equivalent) at appropriate dose with difficulties tapering
 - a. Initial low-dose prednisone 15 mg/day or equivalent
 - i. Lower doses 7.5 mg to 10 mg/day) may be trialed in patients with comorbidities such as diabetes, severe hypertension, or heart failure
 - b.
 - b. A higher dose of glucocorticoid (e.g.; prednisone 25 mg/day) then reassessed in 4 to 6 weeks
 - c. Relapsing or recurring disease is common during tapering
 - i. If recurrence of PMR occurred > 3 months after complete discontinuation of glucocorticosteroid, re-initiation of glucocorticosteroid can occur
3. For mixed presentations of PMR (for example, PMR and rheumatoid arthritis, elevated ESR/CRP), Member must have failed at least 12 weeks of methotrexate (7.5-10 mg/week)
4. Initiation of sarilumab is not recommended in patients with ANC < 2,000/mm³, platelets < 150,000/mm³ or liver transaminases > 1.5 times ULN.

5. Member will not be using sarilumab in combination with another targeted specialty medications (e.g., adalimumab, infliximab, etanercept, certolizumab, golimumab, abatacept, tofacitinib, Upadacitinib, list not all inclusive).

HIDRADENITIS SUPPURATIVA

Hidradenitis Suppurativa (HS) is a chronic, debilitating, inflammatory disease that causes skin abscesses and scarring on the skin. Disease severity is often categorized as mild, moderate, or severe. Examples of objective measures to assess disease severity include Hurley staging, Sartorius score, Physician Global Assessment, and Hidradenitis Suppurativa Severity Index. Guidelines recommend consideration of biologic medication for those with moderate to severe disease who do not respond to conventional therapy. Regardless of the severity, the goal of HS treatment is to reduce pain, drainage, scarring, and the intensity and frequency of flares.

1. Member has a clinical diagnosis of moderate to severe hidradenitis suppurativa as defined by having Hurley Stage II and/or Stage III disease severity (Stage II – Recurrent abscesses with sinus tracts and scarring, single or multiple widely spread lesions; Stage III – Diffuse or almost diffuse involvement, or multiple interconnected sinus tracts and abscesses across the entire area).
2. Non-smoker OR evidence of smoking cessation.
3. Trial and failure of prior therapies in THREE different treatment categories. EXAMPLES: (a) intralesional steroids (e.g., triamcinolone); (b) topical antibiotics (e.g., clindamycin); (c) systemic antibiotic (e.g., doxycycline, minocycline, clindamycin, erythromycin, cephalosporin); (d) oral retinoids (e.g., isotretinoin, acitretin); (e) hormonal therapy.
4. Failure of laser therapy or clinical rationale as to why laser therapy is not medically indicated or appropriate. Laser therapy can be used together with medical therapy. For example, use of Nd:YAG laser destroys hair follicles that cause HS, and also collapses the sinus tracts and nodules. Laser therapy has been shown to be helpful in decreasing the severity of mild to moderate HS and preventing new lesions from occurring.
5. Biologic therapy for coverage will include HAP preferred anti-TNF agents with a prevalence of supportive data (infliximab and adalimumab) if HAP criteria are met
6. In addition to criteria outlined above, for Secukinumab:
 - a. Member must fail a therapeutic trial (including high-dose infliximab (10 mg/kg every 4 weeks) and adalimumab weekly) of ALL HAP preferred anti-TNF agents (unless clinically significant medical contraindications are documented).
 - b. Dose escalation of secukinumab to 300 mg every 2 weeks is not covered. Add-on supportive, standard of care measures can be used along with standard dosing of secukinumab 300 mg every 4 weeks (maintenance).

PSORIATIC ARTHRITIS

Psoriatic Arthritis (PsA) is a chronic inflammatory arthropathy that affects up to 40% of patients with skin or nail psoriasis. It is categorized as a seronegative spondyloarthritis, causing arthritis, enthesitis, dactylitis and axial inflammation. Patients with extensive skin psoriasis should be evaluated by a dermatologist. Treatment options include traditional and biological DMARDs and response to therapy should consider both skin and joint disease activity. Close collaboration between rheumatology and dermatology specialists is desired for optimal management.

1. Member is diagnosed with Psoriatic Arthritis (PsA) as established by joint inflammation, absence of rheumatoid factor, and typical psoriatic skin and nail lesions.
 - a. If Member is diagnosed with **axial disease** (affected joints may include spine, hips and shoulders): Member has failed therapeutic trials with at least two NSAIDs, including celecoxib, of at least 3 months duration or has intolerable side effects or contraindications to treatment with NSAIDs.
 - b. If Member is diagnosed with **peripheral disease** (affected joints may include arms, legs, elbows, wrists, hands and feet): Member has failed a therapeutic trial, of at least 3 months duration, with at least 2 conventional DMARDs (methotrexate, leflunomide, sulfasalazine, cyclosporine) or has intolerable side effects or contraindications to treatment with DMARDs.
 - i. Members with adverse prognostic factors defined as 5 or more swollen joints with elevated C reactive protein (CRP) persisting for more than three months, and/or structural joint damage due to disease, and/or previous use of systemic corticosteroids may be considered for HAP preferred anti-TNF therapy after a therapeutic trial, of at least 3 months duration, with 1 conventional DMARD.
2. Unless otherwise noted on the Formulary as covered without a prior authorization, coverage of HAP preferred anti-TNF medications (e.g., etanercept, list not all inclusive) require coverage criteria of diagnosis (as outlined above) to be met.
3. For Certolizumab pegol, Golimumab, Secukinumab, Abatacept, Guselkumab or Ustekinumab, all of the following apply:
 - a. In addition to diagnosis criteria outlined above, Member must fail a therapeutic trial (at least 3 months duration) of:
 - i. All HAP preferred anti-TNF agents or have intolerable side effects or contraindications to treatment with all HAP preferred anti-TNF agents
 - ii. Apremilast (Specific prior authorization coverage applies). Refer to Pharmacy Care Management HAP Criteria for Use of Drugs Document.
 - iii. Upadacitinib (Rinvoq) – Specific Prior Authorization coverage applies. Refer to Pharmacy Care Management HAP Criteria for Use of Drugs Document.
 - iv. Tofacitinib (Xeljanz) – Specific Prior Authorization coverage applies. Refer to Pharmacy Care Management HAP Criteria for Use of Drugs Document.

- b. The most cost-effective treatment course (taking into consideration requested indication, dosing, and weight) among the specialty medications (intravenous or subcutaneous) on Formulary will be approved. Dosing will follow FDA approved dosing regimen unless otherwise noted with dosing limitations that apply:
 - i. For Secukinumab:
 - A. IV: 1.75 mg/kg every 4 weeks (max. maintenance dose 300 mg per infusion). Initiation with a loading dose is not covered.
 - B. PsA with coexistent moderate to severe plaque psoriasis: Subcutaneous dosing will follow FDA approved dosing regimen.
 - C. PsA without coexistent moderate to severe plaque psoriasis: Cosentyx SQ 150 mg every 4 weeks (without a loading dose). Initiation with a loading dose is not covered.
 - ii. For Ustekinumab: After initial induction of ustekinumab, dosing of 45 mg every 12 weeks. For patients over 100 kg, if 45 mg dose is showing clinical improvement, but not remission after 12 weeks of treatment: 90 mg every 12 weeks. After induction, dosing intervals less than every 12 weeks (maintenance phase) are not approved.

RHEUMATOID ARTHRITIS:

Goals of **rheumatoid arthritis (RA)** treatment include prevention of joint damage, preservation of function, and decrease in pain with the ultimate goal of disease remission. Initial management begins with establishing the diagnosis, performing a baseline evaluation, and determining the prognosis. Initial symptom control may be achieved by nonsteroidal anti-inflammatory drugs (NSAIDs), glucocorticoid joint injection, and/or low-dose prednisone. However, the initiation of disease-modifying antirheumatic drug (DMARD) therapy should not be delayed beyond 3 months for any patient with an established diagnosis, who despite NSAID treatment, has ongoing joint pain, significant morning stiffness or fatigue, active synovitis, persistent elevation of the ESR or CRP level, or radiographic joint damage. DMARDs have the potential to reduce or prevent joint damage and preserve joint integrity and function. The traditional DMARDs used in RA include hydroxychloroquine (HCQ), leflunomide, methotrexate (MTX), minocycline, sulfasalazine (SSZ), D-penicillamine (D-Pen), and gold salts. Biological DMARDs include abatacept, adalimumab, certolizumab pegol, etanercept, infliximab, golimumab, rituximab, tocilizumab, tofacitinib citrate and anakinra. **Felty's syndrome (FS)** is a subset of seropositive RA complicated by granulocytopenia and splenomegaly. Nonbiologic DMARDs and glucocorticoids are used to treat FS, but biologic DMARDs, including anti-TNFs, have not proven effective. Biologic DMARDs may be used for RA.

1. Member is diagnosed with RA as established by ACR/EULAR (2010) Classification Criteria for RA, with ONE of the following:
 - a. Member is diagnosed with early RA (disease duration < 6 months) and has high disease activity and poor prognostic factors (e.g. functional limitation, extra-articular disease, positive rheumatoid factor or bony erosions by radiograph), OR
 - b. Member has established RA (disease duration > 6 months) and has moderate or high disease activity.
2. Member must meet the criteria of trial of triple therapy:
 - a. Member must fail a therapeutic trial of triple therapy with oral non-biologic DMARDs, a combination of methotrexate, hydroxychloroquine and sulfasalazine, of at least 3 months duration or have intolerable side effects or contraindications to triple therapy.
 - i. Therapeutic trial of triple therapy is accepted as methotrexate 25 mg per week, hydroxychloroquine 200 mg twice daily, and sulfasalazine 1000 mg twice daily, unless dose is limited by intolerable side effects.
 - ii. Leflunomide may be substituted for methotrexate for Members with intolerable side effects or contraindications specific to methotrexate.
 - iii. If triple therapy is not feasible due to drug allergy or intolerable side effects with one of the medication components of triple therapy that cannot be substituted; then failure to dual or combination DMARD regimen is required.
3. Unless otherwise noted on the Formulary as covered without a prior authorization, coverage of HAP preferred anti-TNF medications (e.g.; etanercept, list not all inclusive) require coverage criteria 1 and 2 to be met.
4. For Tocilizumab or Sarilumab all of the following apply:
 - a. Member must meet the criteria of diagnosis and trial of triple therapy (see above):
 - b. Member must fail a therapeutic trial of ALL HAP preferred anti-TNF agents (unless clinically significant medical contraindications are documented).
 - i. Failure of TNF inhibitor treatment is defined as a minimum of three months trial with <50% improvement from baseline assessment or new disease activity.
 - c. The most cost-effective regimen is required: preference given to Tocilizumab 162mg every other week prior to trial of Sarilumab.
 - i. Dosing limitation for Tocilizumab: Adults weighing less than 100 kg, initial dosing shall be 162 mg every other week. If 162 mg every other week dose is not showing adequate clinical response after 12 weeks of treatment: may increase to 162 mg every week.
 - d. Member must continue weekly methotrexate or other non-biologic DMARDs.
5. For Certolizumab pegol, Golimumab, Anakinra, or Abatacept all of the following apply:
 - a. Member must meet criteria of diagnosis and trial of triple therapy (see above) AND
 - b. Member must fail a therapeutic trial of ALL HAP preferred anti-TNF agents (unless clinically significant medical contraindications are documented).

- i. Failure of TNF inhibitor treatment is defined as a minimum of three months trial with <50% improvement from baseline assessment or new disease activity.
- c. Member must fail a therapeutic trial, of at least 3 months duration or have intolerable side effects or contraindications to treatment to all the following:
 - i. Upadacitinib (Rinvoq) and Tofacitinib (Xeljanz) – Specific Prior Authorization coverage applies. Refer to Pharmacy Care Management HAP Criteria for Use of Drugs Document.
 - ii. Tocilizumab
 - iii. Rituximab
 - iv. Sarilumab
 - v. For Golimumab: Member must continue weekly methotrexate.

FELTY'S SYNDROME:

- 1. Refer to section for Rheumatoid arthritis above; members will be evaluated based on their RA diagnosis, not severity of granulocytopenia and other findings specific to FS.

JUVENILE IDIOPATHIC ARTHRITIS

Juvenile idiopathic arthritis (JIA)/ERA is defined by the International League of Associations for Rheumatology (ILAR) as arthritis of unknown etiology that begins before the sixteenth birthday and persists for at least 6 weeks with other known conditions excluded. JIA often persists into adulthood and can result in significant long-term morbidity, including physical disability. JIA is classified into the following categories, but the categories do not strictly determine treatment choice: history of arthritis of 4 or fewer joints, history of arthritis of 5 or more joints, active sacroiliac arthritis, systemic arthritis with active systemic features (and without active arthritis), and systemic arthritis with active arthritis (and without active systemic features). Enthesitis-related arthritis (ERA) is one of subtypes of juvenile idiopathic arthritis (JIA). ERA is classified as children with arthritis and enthesitis or arthritis/enthesitis with at least two of the following features: onset of arthritis in males > 6 years of age, sacroiliac joint tenderness, inflammatory back (lumbosacral) pain, presence of HLA B27, acute symptomatic anterior uveitis, or history of seronegative spondyloarthropathies (acute anterior uveitis, ankylosing spondylitis, inflammatory bowel disease with sacroiliitis, or reactive arthritis). Treatment options include traditional and biological DMARDs.

1. History of Arthritis of 4 or more joints:

- a. For Tocilizumab:
 - i. Member must fail a therapeutic trial of methotrexate of at least 3 months duration.
 - A. For Members who have intolerable side effects or contraindications to methotrexate, leflunomide may be an appropriate treatment alternative
 - B. Member must fail a therapeutic trial of all HAP preferred anti-TNF agents of at least 4 months duration or have intolerable side effects or contraindications to treatment with anti-TNF agents.
- b. For Abatacept:
 - i. Member must fail a therapeutic trial of at least 3 months duration to the following, AND
 - A. Methotrexate (or leflunomide)
 - B. All HAP preferred anti-TNF agents of at least 4 months duration or have intolerable side effects or contraindications to treatment with anti-TNF agents.
 - C. Tocilizumab
 - D. Tofacitinib (Xeljanz) – Specific Prior Authorization coverage applies. Refer to Pharmacy Care Management HAP Criteria for Use of Drugs Document.

2. Active sacroiliac arthritis:

- a. For Etanercept, the Member must meet one of the following:
 - i. Failure of a therapeutic trial of methotrexate or sulfasalazine of at least 3 months duration, or
 - ii. Failure of a therapeutic trial of NSAIDs of at least 3 months duration with high disease activity and features of a poor prognosis.
 - iii. Discussion with provider regarding selection of HAP preferred anti-TNF agents.

3. Systemic Arthritis with Active Systemic Features (ex. fever, elevated inflammatory markers) and without Active Arthritis:

- a. For Anakinra:
 - i. Member must fail a therapeutic trial of systemic glucocorticoids of at least 3 months duration.

4. Systemic Arthritis with Active Arthritis:

- a. For Etanercept:
 - i. Member must fail a therapeutic trial of methotrexate of at least 3 months duration or have intolerable side effects or contraindications to methotrexate.
 - ii. Discussion with provider regarding selection of HAP preferred anti-TNF agents.
- b. For Abatacept, Tocilizumab or Anakinra:
 - i. Member must fail a therapeutic trial of methotrexate of at least 3 months duration, AND
 - ii. Member must fail a therapeutic trial of all HAP preferred anti-TNF agents of at least 4 months duration or have intolerable side effects or contraindications to treatment with anti-TNF agents.

5. Enthesitis-Related Arthritis:

- a. For Etanercept and Secukinumab:

- i. If Member has ERA with axial disease or peripheral disease, Member has failed therapeutic trials with at least two non-steroidal anti-inflammatory drugs (NSAIDs) of at least 1 month of duration or has intolerable side effects or contraindications to treatment with NSAIDs.
- ii. If Member has ERA peripheral disease, Member has failed a therapeutic trial, of at least 3 months duration, with at least 1 conventional DMARDs (methotrexate, sulfasalazine) or has intolerable side effects or contraindications to treatment with DMARDs.
- iii. For Etanercept: Discussion with provider regarding selection of HAP preferred anti-TNF agents.
- iv. For Secukinumab: Member must fail a therapeutic trial of all HAP preferred anti-TNF agents of at least 4 months duration or have intolerable side effects or contraindications to treatment with anti-TNF agents.

ANKYLOSING SPONDYLITIS

Ankylosing spondylitis (AS) is a subtype of spondyloarthritis that presents clinically with spinal (axial, including sacroiliitis) and/or peripheral joint inflammation (which may include enthesopathy and dactylitis). AS is often associated with the HLA-B27 gene; 90% of western European Caucasians with AS test positive for HLA-B27 vs. 8% without AS and 48% of African Americans with AS test positive for HLA-B27 vs. 2-4% without AS. Treatment options for AS include NSAIDs, sulfasalazine and anti-TNFs.

1. Member is diagnosed with Ankylosing Spondylitis as established by:
 - a. Bilateral sacroiliitis $\geq$ grade 2 OR unilateral sacroiliitis $\geq$ grade 3
 - b. One or more of the following:
 - i. Inflammatory low back pain ($\geq$ 3 months) that does not improve with rest
 - ii. Limited lumbar spine motion
 - iii. Decreased chest expansion for age and sex
2. Member must meet one of the following:
 - a. Member is diagnosed with **axial** disease (affected joints may include spine, hips and shoulders) and has failed therapeutic trials with at least two NSAIDs, including celecoxib, of at least 3 months duration or has intolerable side effects or contraindications to treatment with NSAIDs.
 - b. Member is diagnosed with **peripheral** disease (affected joints may include arms, legs, elbows, wrists, hands and feet) and has failed a therapeutic trial with sulfasalazine of at least 4 months duration or has intolerable side effects or contraindications to treatment with sulfasalazine.
3. Unless otherwise noted on the Formulary as covered without a prior authorization, coverage of HAP preferred anti-TNF medications (e.g.; etanercept, list not all inclusive) require coverage criteria 1 and 2 to be met.
4. For Secukinumab, Certolizumab pegol, or Golimumab, in addition to diagnosis criteria outlined above, Member must fail a therapeutic trial, of at least 3 months duration, of the following:
 - a. All HAP preferred anti-TNF agents or have intolerable side effects or contraindications to treatment with these preferred anti-TNF agents.
 - b. Upadacitinib (Rinvoq) – Specific Prior Authorization coverage applies. Refer to Pharmacy Care Management HAP Criteria for Use of Drugs Document.
 - c. Tofacitinib (Xeljanz) – Specific Prior Authorization coverage applies. Refer to Pharmacy Care Management HAP Criteria for Use of Drugs Document.
5. Dosing limitation: The most cost-effective treatment course (taking into consideration requested indication, dosing, and weight) among the specialty medications (intravenous or subcutaneous) on Formulary will be approved. Dosing will follow FDA approved dosing regimen unless otherwise noted with dosing limitations that apply:
 - a. For Secukinumab:
 - i. IV 1.75 mg/kg every 4 weeks. Initiation with a loading dose is not covered.
 - ii. SQ: 150 mg every 4 weeks (initiation with a loading dose is not covered). If active ankylosing spondylitis persists after 12 weeks of 150 mg every 4 weeks, dose may be increased to secukinumab 300 mg every 4 weeks.

GIANT CELL ARTERITIS

Giant Cell Arteritis (temporal arteritis) is the most common systemic vasculitis in North America of older adults. The specific cause of blood vessel inflammation is unknown; and can affect the large blood vessels of the scalp, neck and arms. Symptoms include headaches, jaw pain, vision loss, fever, and fatigue. An established diagnosis may include positive biopsy or imaging.

1. Member is diagnosed with Giant Cell Arteritis via positive biopsy and imaging studies
2. For Tocilizumab, Member must have trialed at least 12 weeks of glucocorticosteroids (e.g. prednisone or equivalent) at appropriate dose.
3. In clinical trials, with prednisone taper, tocilizumab 162 mg weekly and 162 mg every other week both showed superiority in achieving sustained remission from Week 12 through Week 52.
 - a. New starts limited to tocilizumab 162 mg every other week.
 - b. If after 12 weeks of 162 mg every other week with clinical improvement but no remission, consideration for dose escalation to tocilizumab 162 mg weekly.

UVEITIS

Uveitis is a form of eye inflammation that affects the uvea (middle layer of tissue in the eye wall). Uveitis may result from infection, injury, an autoimmune disease or inflammatory disease. Common warning signs include eye redness, pain and blurred vision.

1. Member has a clinical diagnosis of non-infectious uveitis (including intermediate, posterior, and panuveitis).
2. Evidence of chronic, recurrent, treatment-refractory or vision-threatening disease.
3. Prior failure of periocular, intraocular, or systemic corticosteroids.
4. Prior failure of TWO immunosuppressive medications (e.g., methotrexate, mycophenolate, azathioprine, cyclophosphamide, or cyclosporine).
5. Concurrent immunosuppressive medication or prednisone to help control the disease (in clinical trials, patients were administered concurrent prednisone therapy).
6. Biologic therapy for coverage will include HAP preferred anti-TNF agents with a prevalence of supportive data (infliximab and adalimumab).

SYSTEMIC SCLEROSIS-ASSOCIATED INTERSTITIAL LUNG DISEASE

Systemic sclerosis-associated interstitial lung disease (SSc-ILD) is characterized by inflammation and progressive scarring of the lungs. SSc-ILD consists of histopathologic subtypes such as nonspecific interstitial pneumonitis and usual interstitial pneumonitis.

For Tocilizumab:

1. Medication is prescribed by or in consultation with a pulmonologist or a rheumatologist.
2. Member is 18 years and older
3. Systemic sclerosis-associated interstitial lung disease (ILD) - diagnosed with SSc-ILD based upon the 2013 American College of Rheumatology / European League Against Rheumatism classification criteria for SSc with onset of disease (first non-Raynaud symptom) of less than 5 years and greater than or equal to 10% fibrosis on a chest high resolution computed tomography (HRCT) scan conducted within the previous 12 months
4. Member has elevated acute phase reactants with at least one of the following:
 - a. C-reactive protein (CRP) $\geq$ 6 mg/mL
 - b. Erythrocyte sedimentation rate (ESR) $\geq$ 28 mm/h
 - c. Platelet count $\geq$ 330 x 10⁹/L
5. Forced vital capacity (FVC) is $\geq$ 55% of the predicted value
6. Baseline percent predicted diffusing capacity of the lungs for carbon monoxide (%DLCO, corrected for hemoglobin) $>$ 45%
7. After 12 weeks of use, decline in FVC, increased fibrotic changes on imaging, or worsening of symptoms despite treatment with drugs commonly used in clinical practice to treat ILD – e.g. mycophenolate or cyclophosphamide
 - a. Based on evidence of comparable efficacy and favorable safety profile, HAP favors trial of mycophenolate 1.5 to 3 grams daily (in divided doses)
8. Concurrent use with other specialty treatments is not covered (e.g., nintedanib, pirfenidone)

FAMILIAL MEDITERRANEAN FEVER

Familial Mediterranean Fever (FMF) is a hereditary autoinflammatory genetic disorder. It is caused by mutations in the MEFV gene, which leads to inappropriate activation of the immune system, resulting in recurrent episodes of fever and inflammation. FMF is characterized by acute attacks of serositis— inflammation of the linings of the chest (pleuritis), abdomen (peritonitis), and joints (synovitis)—leading to severe pain.

General criteria:

1. Medication is prescribed by or in consultation with a rheumatologist, nephrologist, geneticist
2. Member is FDA approved age and weight for use
3. Member must have a confirmed diagnosis of FMF based on the following:
 - a. Genetic testing confirmatory of two pathogenic mutations in the MEFV gene OR one pathogenic mutation and member is responsive to a 6-month trial of colchicine therapy; AND
 - b. Meets diagnosis criteria according to the Livneh Criteria (for adults) or Turkish Pediatric Criteria (for children): <https://ard-bmj-com.sladenlibrary.hfhs.org/content/74/4/635.long>

For Etanercept

1. Member meets general criteria 1-3; AND
2. Member has tried and failed ALL of the following (unless clinically significant medical contraindications are documented) defined as persistent attacks ($>$ 1 attack per month) or persistent inflammation:
 - a. Colchicine titrated to maximum doses (up to 3 mg for adults, 2 mg for children) for at least 6-months consistently
 - i. For members who are unable to tolerate higher doses of colchicine with once-daily dosing due to side effects, a divided dosing regimen (twice daily) has been trialed

- b. For members with accompanied chronic arthritis, colchicine in combination with ONE immunomodulator (e.g., methotrexate, azathioprine, sulfasalazine) for at least 3 months
- c. Infliximab for at least 3 months
- d. Adalimumab for at least 3 months

For Anakinra

- 1. Member must meet all the general criteria(1-3)
- 2. Member has tried and failed ALL of the following (unless clinically significant medical contraindications are documented) defined as persistent attacks (>1 attack per month) or persistent inflammation:
 - a. Colchicine titrated to maximum doses (up to 3 mg for adults, 2 mg for children) for at least 6-months consistently
 - i. For members who are unable to tolerate higher doses of colchicine with once-daily dosing due to side effects, a divided dosing regimen (twice daily) has been trialed
 - b. For members with accompanied chronic arthritis, colchicine in combination with ONE immunomodulator (e.g., methotrexate, azathioprine, sulfasalazine) for at least 3 months
 - c. Colchicine in combination with all HAP preferred anti-TNF medications for at least 3 months

For Canakinumab:

- 1. Member must meet all the general criteria (1-3)
- 2. Member has tried and failed ALL of the following (unless clinically significant medical contraindications are documented) defined as persistent attacks (>1 attack per month) or persistent inflammation:
 - a. Colchicine titrated to maximum doses (up to 3 mg for adults, 2 mg for children) for at least 6-months consistently
 - i. For members who are unable to tolerate higher doses of colchicine with once-daily dosing due to side effects, a divided dosing regimen (twice daily) has been trialed
 - b. For members with accompanied chronic arthritis, colchicine in combination with ONE immunomodulator (e.g., methotrexate, azathioprine, sulfasalazine) for at least 3 months
 - c. Colchicine in combination with all HAP preferred anti-TNF medications at maximum doses for at least 3 months
 - d. Anakinra

ADULT ONSET STILL'S DISEASE

Adult Onset Still's Disease (AOSD) is a rare, systemic inflammatory disorder characterized by high spiking fevers, arthritis, and a distinctive salmon-colored rash. It is an autoinflammatory disease, meaning it involves abnormal activation of the immune system without an external trigger. Elevated inflammatory markers such as C-reactive protein (CRP) and ferritin are often seen during flare-ups, aiding in diagnosis. The disease can range from self-limiting episodes to chronic forms that lead to significant joint destruction and systemic complications, including macrophage activation syndrome (MAS). Features concerning for macrophage activation syndrome (MAS)/hemophagocytic lymphohistiocytosis (HLH), including high levels of inflammation, high ferritin, cytopenias, and transaminitis

- 1. Medication is prescribed by or in consultation with a rheumatologist
- 2. Member is FDA approved age and weight for use
- 3. Member has a definitive diagnosis of AOSD with persistent fever (at least 39C (102.2 F) spiking once to twice per day lasting at least one week) without a clear source plus any TWO of the following:
 - a. Evanescent, salmon-colored, nonpruritic, macular or maculopapular rash on the trunk and extremities during febrile episodes
 - b. Arthralgia and/or arthritis lasting two weeks or longer
 - c. Leukocytosis (10,000/microL) with at least 80 percent granulocytes
 - d. Markedly elevated serum ferritin (>3000 ng/mL)
 - e. Markedly elevated CRP and/or ESR
- 4. Member has been evaluated and the follow differential diagnoses have been ruled out: infectious disease (e.g., acute viral infection), malignancy (e.g., lymphoproliferative disorders), drug reactions (e.g., DRESS), systemic autoimmune rheumatic diseases (e.g., RA, SLE), autoinflammatory diseases, neutrophilic dermatoses (e.g., Sweet Syndrome).
- 5. For Entercept:
 - a. Member has definitive AOSD without MAS and documentation of treatment failure or labeled contraindication to ALL of the following:
 - i. NSAIDs (naproxen 500mg BID, ibuprofen 800mg TID, Indomethacin 25-50 mg TID).
 - ii. Glucocorticosteroids (prednisone 10-30 mg once daily) for at least 2-months
 - iii. At least 3-month trial of ONE DMARDs such as methotrexate, cyclosporine, or leflunomide;
- 6. For Tocilizumab
 - a. Member has definitive AOSD without MAS and documentation of treatment failure or labeled contraindication to ALL of the following:
 - i. NSAIDs (naproxen 500mg BID, ibuprofen 800mg TID, Indomethacin 25-50 mg TID).
 - ii. Glucocorticosteroids (prednisone 10-30 mg once daily) for at least 2-months
 - iii. At least 3-month trial of ONE DMARDs such as methotrexate, cyclosporine, or leflunomide;
 - iv. At least 3-month trial of all HAP preferred anti-TNF inhibitors
- 7. For Anakinra

- a. Member has newly diagnosed AOSD with active MAS and will be used concurrently with glucocorticoids
 - i. 4-week trial of high-dose corticosteroids (e.g., prednisone methylprednisolone)
- b. Member has definitive AOSD without MAS and has documentation of treatment failure or labeled contraindication to ALL of the following:
 - i. NSAIDs (naproxen 500mg BID, ibuprofen 800mg TID, Indomethacin 25-50 mg TID).
 - ii. Glucocorticosteroids (prednisone 10-30 mg once daily) for at least 2-months
 - iii. At least 3-month trial of ONE DMARDs such as methotrexate, cyclosporine, leflunomide;
 - iv. At least 3-month trial of the following HAP preferred biologics:
 - A. HAP preferred anti-TNFs: infliximab (Renflexis, Inflectra), adalimumab (Hadlima);
 - B. Tocilizumab;

8. For Canakinumab

- a. Member has definitive AOSD with or without MAS symptoms.
- b. Member must have documentation of treatment failure or label contraindications to ALL of the following:
 - i. NSAIDs (naproxen 500mg BID, ibuprofen 800mg TID, Indomethacin 25-50 mg TID).
 - ii. Glucocorticosteroids (prednisone 10-30 mg once daily) for at least 2-months
 - iii. At least 3-month trial of ONE DMARDs such as methotrexate, cyclosporine, leflunomide;
 - iv. At least 3-month trial of ALL HAP preferred biologics at maximum doses:
 - A. Adalimumab (Hadlima);
 - B. Infliximab (Renflexis, Inflectra);
 - C. Etanercept;
 - D. Tocilizumab;
 - E. Rituximab;
 - F. Anakinra;
- c. Failure to control symptoms despite appropriate combination therapy (e.g., Anakinra + cyclosporine)

LIMITATIONS

1. In addition to application of coverage criteria, HAP reserves the right to select the most cost-effective treatment course (taking into consideration requested indication, dosing, and weight) among the specialty medications (intravenous or subcutaneous) on Formulary. [Coverage consideration is given to HAP preferred medications prior to coverage of other specialty medications.]
2. Requests for dose escalation will be examined according to evidence for efficacy of proposed regimen, add-on non-specialty medications to help manage symptoms, cost and clinical setting. Only one dose escalation change (most cost-effective escalation) at a time will be considered if supported by compendia. Requests for a dose increase and concurrent shorter-interval of administration will not be approved. If Member is showing clinical improvement, but not remission after 12-week trial of dose escalation change, further modification to therapy or change in therapy will be considered.
3. Coverage is limited to FDA approved dosing regimens for the requested indication (unless otherwise noted by dosing limitations for a requested indication within the Coverage Criteria).
 - a. Dose intensification will be reviewed on a case-by-case basis and take into consideration the following:
 - i. No response or loss of response to a minimum of 3 months of a standard maintenance regimen
 - ii. Oral DMARD medications (e.g. methotrexate) have been used concurrently for 3 months with a standard maintenance regimen
 - iii. Alternative treatment options have been explored
4. Approval is granted for ONE specialty medication (injectable or oral) for a given Member at one time. (For example: Apremilast (oral) medication or upadacitinib (oral) medication is not approved in combination with injectable biologic therapy in active psoriatic arthritis.)
5. Requests for initial authorization for Members who were established on therapy via the receipt of a manufacturer supplied sample or any form of assistance from a manufacturer sponsored program are required to meet initial authorization criteria as if the Member was new to therapy.
6. Authorization requests for specialty medications listed above:
 - a. Initial approval will be limited to a 6-month period.
 - b. Subsequent approvals may be limited to 3 to 6 months for the following reasons:
 - i. Inadequate response to treatment
 - ii. Inconsistent pattern of medication adherence
 - iii. Safety concern
 - iv. Off-label dosing
7. In general, if renewal requirements are satisfied, subsequent approvals of 12 months duration are contingent on demonstrated response as evident in the medical records. Exceptions to the renewal duration are: change in dose, lack of response, lack of adherence to the regimen and lack of follow-up with the specialty provider.
8. **Plaque psoriasis:** Continuation coverage is limited to Members who demonstrate clearing of 50% or greater of affected body surface area of plaque psoriasis
9. **Rheumatoid arthritis:** Continuous coverage for rheumatoid arthritis is limited to Members who demonstrate an appropriate response after 3 months of therapy.
 - a. Appropriate response (ACR 20) is defined as:
 - i. $\geq 20\%$ improvement in the number of tender joints AND

- ii. $\geq 20\%$ improvement in the number of swollen joints AND
 - iii. $\geq 20\%$ improvement in 3 of the following 5 measures:
 - A. Patient pain assessment
 - B. Patient global assessment
 - C. Physician global assessment
 - D. Patient self-assessed disability
 - E. Acute phase reactant (ESR or CRP)
 - iv. OR achieved an equivalent therapeutic response as indicated by scoring using an RA disease activity scale outlined in Table below.
- 10. **Juvenile idiopathic arthritis:** Continuous coverage for juvenile idiopathic arthritis is limited to Members who demonstrate an appropriate response after 3 months of therapy.
 - a. Appropriate response is defined as
 - i. $\geq 30\%$ improvement in ≥ 3 of the following categories, with no more than 1 of the remaining categories worsened by $>30\%$:
 - A. Physician global assessment of disease activity
 - B. Parent/patient global assessment of overall well-being (each scored on a 10-cm VAS)
 - C. Functional ability
 - D. Number of joints with active arthritis
 - E. Number of joints with limited range of motion
 - F. ESR
- 11. **Ankylosing spondylitis:** Continuous coverage for ankylosing spondylitis is limited to Members who demonstrate an appropriate response after 3 months of therapy.
 - a. Appropriate response is defined as (ASAS20 response)
 - i. $>20\%$ improvement from baseline and absolute baseline improvement of >10 (on a 0-100 mm scale) in at least 3 of the following 4 domains:
 - A. Patient global assessment
 - B. Pain
 - C. Function (BASFI)
 - D. Inflammation
 - ii. Absence of deterioration from baseline ($>20\%$ and absolute change of at least 10 on a 0-100 mm scale) in the potential remaining domain
- 12. **Psoriatic arthritis:** Continuous coverage for psoriatic arthritis is limited to Members who demonstrate an appropriate response after 3 months of therapy.
 - a. Appropriate response (PsARC) is defined as improvement in two factors (with at least one being a joint score) with worsening of none of the following four factors:
 - i. Patient's global assessment (on a 0-5 Likert scale, improvement defined as decrease of 1 category, worsening defined as increase of 1 category)
 - ii. Physician global assessment (as above)
 - iii. Tender joint count (improvement defined as reduction by at least 30%, worsening defined as an increase of at least 30%)
 - iv. Swollen joint count
 - b. Appropriate response may also be assessed using ACR 20 (see section 2. a) above)
- 13. **Giant Cell Arteritis:** Continuous coverage for giant cell arteritis (GCA) is limited to Members who demonstrate appropriate response after 3 months of therapy.
 - a. Appropriate response is defined by the following examples:
 - i. Absence of GCA signs and symptoms
 - ii. Normalization of erythrocyte sedimentation rate (ESR)
 - iii. Normalization of C-reactive protein (CRP)
 - iv. Successful prednisone taper (may take up to 26 weeks)
- 14. **Systemic sclerosis-associated interstitial lung disease:** Continuous coverage for SSc-ILD is limited to Members who demonstrate appropriate response after 6 months of therapy. Appropriate response is defined by the following examples:
 - a. Disease response by a reduction in the rate of decline or stabilization in forced vital capacity (%FVC) or percent predicted FVC (ppFVC) as compared to pre-treatment baseline; AND
 - b. Member does not have evidence of disease progression defined as an absolute decline of more than 10% in percent-predicted FVC within any 12-month period
- 15. **Generalized Pustular Psoriasis:**
 - a. Flares: Initial approval is limited to single dose (900mg IV) of Spesolimab-sbzo . Authorization is for -1-week.
 - i. Patients 40-45 kg doses will be limited to 10 mg/kg
 - ii. Limit of two doses per single GPP flare. Requests for the second dose for treatment of the same GPP flare require all of the following:
 - A. Member has already received one initial dose of Spesolimab-sbzo for a current GPP flare; and
 - B. New documentation supports a second dose (within 7 days of the previous dose) of Spesolimab-sbzo in order to treat persistent GPP flare symptoms including the following:
 - I. Either a GPPPGA total score ≥ 2 OR GPPPGA pustulation subscore ≥ 2 ; and

- II. One of the following: Fever, asthenia, myalgia, elevated C-reactive protein, or leukocytosis with peripheral blood neutrophilia (above the upper limit of normal [ULN])
 - b. Note: If the Member has been treated with Spesolimab-sbzo for a previous GPP flare, then a new (different) GPP flare may be treated with up to two doses of Spesolimab-sbzo if at least 12-weeks have elapsed since the last dose of Spevigo.
 - c. **Maintenance:** Initial approval is limited to Spesolimab-sbzo 300 mg loading dose, then 150 mg every 12 weeks. Authorization is for 24 weeks. [Pivotal trials showed lower doses were clinically significant in time to first flare.]
 - i. Requests for dose escalation to high dose Spesolimab-sbzo 300 mg every 4 weeks requires evidence of recurrent GPP flare despite adherence to maintenance.
16. **Polymyalgia Rheumatica:** Continuous coverage for PMR is limited to Members who demonstrate appropriate response after 6 months of therapy. Appropriate response is defined by the following examples:
 - a. Serum markers (CRP, ESR) stable or reduced
 - b. Compared to baseline, decreased shoulder, neck, or hip stiffness
 - c. Improved range of motion
17. **Familial Mediterranean Fever:**
 - a. Continuous coverage for FMF is limited to Members who demonstrate appropriate response after 6 months of therapy. Appropriate response is defined by the following examples:
 - i. Serum markers (CRP, ESR) stable or reduced
 - ii. Reduction of recurrent events compared to baseline
 - b. Canakinumab is limited to FDA approved dosing regimens.
 - c. Anakinra is limited to once daily dosing.
18. **Adult-onset Still's Disease:**
 - a. Continuous coverage for AOSD is limited to Member who demonstrate appropriate response after 3 months of therapy. Appropriate response is defined by the following examples:
 - i. Serum markers (CRP, ESR, Ferritin) stable or reduced
 - ii. Compared to baseline, improvement or resolution in fever, rash, swelling, or joint pain
 - b. Canakinumab is limited to FDA approved dosing regimens.
 - c. Anakinra is limited to once daily dosing in maintenance setting.
 - d. Enterecept dosing is limited to 50 mg injections once weekly.
 - e. Tocilizumab is limited to 162 mg injections once weekly and 8 mg/kg intravenously every two weeks.
 - f. For patients in complete remission for at least 3-months, medications should start to be tapered with the goal of discontinuing medications over the next 3-12 months.
 - i. Anakinra: Wean to every other day for 1 month, followed by discontinuation
 - ii. Canakinumab: increase interval from monthly to every other month for two infusions, then to every 3 months for two infusions before discontinuing
 - iii. Tocilizumab:
 - A. Self-administered injections: Space injections from weekly to every other week for 2 months, then monthly for two months before discontinuing.
 - B. Infusions: Space infusions from every two weeks to once monthly for two infusions and then every other month for two infusions before discontinuing.
19. Concurrent alcohol use, as rationale not using conventional disease modifying antirheumatic drug therapy (methotrexate), does not demonstrate medical necessity for coverage of biologics as a criteria exception because alcohol consumption is a modifiable risk factor.
20. Scheduling conflicts/travel barriers are not a contraindication for use, and do not demonstrate medical necessity for not using effective treatment options such as PUVA/PUVB.
21. Billing code J3490 or J3590 (or any other unclassified drugs or biologics code) should not be used for billing of drug products listed in this Policy or to request authorizations in Care Radius. Please refer to New to Market Policy for unclassified drugs.

EXCLUSIONS

1. Immunocompromised (HIV) Members due to the increased risk of opportunistic infection.
2. Members with active tuberculosis; coverage may be considered upon completion of treatment with antitubercular medications.
3. Anti-TNF agents will not be covered for Members with congestive heart failure (NYHA class III/IV) with an ejection fraction of $\leq 50\%$.
4. Tocilizumab is not covered for Members with baseline absolute neutrophil count (ANC) $< 2000/\text{mm}^3$ or platelet count $< 100,000/\text{mm}^3$ or for Members who develop an absolute neutrophil count (ANC) $< 500/\text{mm}^3$ or platelet count $< 50,000/\text{mm}^3$ while on tocilizumab treatment. Laboratory monitoring of neutrophils, platelets, and liver function tests should be performed every 4-8 weeks and lipids every 6 months.
5. Canakinumab (Ilaris®) is not covered for the treatment of juvenile idiopathic arthritis or any other indication not specifically outlined in this benefit policy.

6. Spesolimab-sbzo is not covered for the treatment of drug triggered acute generalized exanthematous pustulosis and is unproven and not medically necessary for the treatment of the following conditions and situations:
- IV administration in excess of 2 doses per single GPP flare
 - Atopic dermatitis
 - Crohn's disease
 - Hidradenitis suppurativa
 - Palmoplantar pustulosis
 - Plaque psoriasis
 - Prevention of GPP flares
 - Ulcerative colitis

Related Benefit Administration Manual Policies:

Please refer to the following BAM policies for related coverage criteria:

- Biosimilar and Preferred Formulations
- Please refer to the Benefit Administration Manual policy: Agents for Neuromyelitis Optica Spectrum Disorders (NMOSD) for information related to rituximab use for NMOSD.
- Please refer to the Benefit Administration Manual policy: Self-Administered Drug Management for Members other than Medicare and HAP Criteria for Use of Drugs document for information related to Risankizumab (Skyrizi).

APPENDIX TABLE 1: DISEASE ACTIVITY MEASURES & GUIDELINES FOR RHEUMATOID CONDITIONS

Instrument	Thresholds of Disease Activity			
	Remission	Low	Moderate	High
American College of Rheumatology Scores (ACR)	Improvement is denoted as either ACR 20, ACR 50 or ACR 70 reflecting either an improvement to the 20%, 50%, or 70% level in the parameters outlined (see section 2.a) under LIMITATIONS)			
Patient Activity Scale (PAS) or PAS-II (range 0-10)	0 - 0.25	0.26 - 3.7	3.71 - <8.0	≥8.0
Routine Assessment of Patient Index Data 3 (RAPID-3) (range 0-10)	0 - 1.0	> 1.0 - 2.0	> 2.0 - 4.0	> 4.0 - 10
Clinical Disease Activity Index (CDAI) (range 0-76.0)	≤ 2.8	> 2.8 - 10.0	> 10.0 - 22.0	> 22
Disease Activity Score in 28 joints (DAS28) (range 0-9.4)	< 2.6	≥ 2.6 - < 3.2	≥ 3.2 - ≤ 5.1	> 5.1
Simplified Disease Activity Index (SDAI) (range 0-86.0)	≤ 3.3	> 3.3 - ≤ 11.0	> 11.0 - ≤ 26	> 26
Children's Health Assessment Questionnaire (CHAQ) (use for age 1 – 19 years)	Higher scores reflect greater disability. The range is 0 –3. 0 Without any difficulty; 1 With some difficulty; 2 With much difficulty; 3 Unable to do. Child or parent marks "Not applicable" if the task is beyond the child's developmental age. –minimum improvement of 0.13 in the CHAQ score to indicate functional improvement			
Juvenile Arthritis Functional Assessment Report (JAFAR) (do not use for age <7 years)	Higher scores reflect greater disability. The range is 0 –2. 0 = able to perform tasks All the time; 1 = able to perform the task sometimes; 2 = able to perform the task almost never.			
Juvenile Arthritis Functional Assessment Scale (JAFAS) (do not use for age <7 years)	Higher scores reflect greater disability. The range is 0 –2. 0 = child completes the task in less than or equal to the criterion time; 1 = child completes the task, but requires longer than the criterion time; 2 = child is unable to complete the task.			
Juvenile Arthritis Functional Status Index (JASI) (validated for children in grades 3-12)	Lower scores reflect greater disability. Item scores. 6 = As well as friends/family without arthritis; 5 = It is a little difficult; 4 = It is very difficult; 3 = Using special equipment; 2 = With a little help from someone; 1 = With a lot of help from someone; 0 = Someone has to do it for me or; 0 = I can't do it because of my arthritis. Potential range for Part I is 0 – 600 (100 items); Potential range for Part II is 0 – 30 (5 items).			
Pediatric Orthopedic Surgeons of North America (POSNA) pediatric musculoskeletal functional health questionnaire (use for age 2 – 18 years)	Higher normative scores reflect better functioning. Scale. Some items use a categorical response format; others use a nominal response (circle "Yes" or imply "No" by not circling "Yes"). Score range. Varies, depending on the type of question; some items employ a 5-point scale, while others use a 4-point scale.			
Bath Ankylosing Spondylitis Disease Activity Index (BASDAI) (range 0-10)	The higher the score, the more severe the patient's disability. >5 = high. 4-5 = moderate. 0 (none or no symptoms) to 10 (very severe symptoms) ≥50% improvement in score is considered a major clinical response			

AS Disease Activity Score (ASDAS-CRP or ASDAS-ESR) Clinically Important Improvement: change ≥ 1.1 Major Improvement: change ≥ 2.0	< 1.3	1.3 < 2.1	2.1 - 3.5	> 3.5
Bath Ankylosing Spondylitis Functional Index (BASFI) (0-100)	The higher the score, the more severe the patient's limitation of function. 0 (no functional impairments) to 10 (maximal impairment)			
Dougados Functional Index (DFI)	0–40, higher values reflecting higher functional impairment			
Bath Ankylosing Spondylitis Metrology Index (BASMI)	The higher the score the more severe the patient's limitation of movement. 0 (normal spinal mobility) to 10 (severely restricted spinal mobility)			
Health Assessment Questionnaire for the spondylarthropathies (HAQ-S)	Responses are 0 = able to do with no difficulty; 1 = able to do with some difficulty; 2 = able to do with much difficulty; and 3 = unable to do. The final score range is 0–3.			
Psoriatic Arthritis Response Criteria (PsARC)	Response is defined as improvement in two factors (with at least one being a joint score) with worsening of none of the following four factors: • Patients global assessment (on a 0]5 Likert scale, improvement defined as decrease of 1 category, worsening defined as increase of 1 category) • Physician global assessment (as above) • Tender joint count (improvement defined as reduction by at least 30%, worsening defined as increase of at least 30%) • Swollen joint count			
Generalized Pustular Psoriasis Physician Global Assessment Score (GPPPGA)	https://academic.oup.com/bjd/article/189/1/138/7131301			

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Benefit Administration Manual Policies are developed to provide guidance to Members and Providers. This Policy relates only to the services or supplies described in it. The existence of a Benefit Administration Manual Policy is not an authorization, certification, explanation of benefits or a contract for the service (or supply) that is referenced in the Policy. Coverage of services for Members is based on the Member's subscriber documents and is subject to all terms and conditions including specific exclusions and limitations. This type of document includes the following: Subscriber contract and associated riders; Member Benefit Guide; or an Evidence of Coverage document (for Medicare Plan Members).

HAP HMO/POS and AHL EPO/PPO Members:

If there is a discrepancy between this policy and coverage described in the subscriber documents, the Member's subscriber documents will apply.

ASO Members:

Coverage as discussed in this policy may not apply to employer groups that are self-funded (referred to as an ASO group [Administrative Services Only]). Each ASO group determines the coverage available to their members which is found in the ASO Benefit Guide and associated riders. If a member has coverage for the type of service covered by this policy, then the medical criteria as discussed in this policy applies to those services.

Medicare Plan Members:

Coverage is based on Medicare (CMS) regulations and guidelines which include the NCDs (National Coverage Decision) and LCDs (Local Coverage Decision) for our area. This Policy does not apply to Medicare Plan members.

EFFECTIVE DATE

09/09/2015

REVISED DATE

03/01/2025

REVIEWED DATE

01/25/2024

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